

Name : Ms. SAKSHI W-O ANUS

Lab No. : 499905246

Ref By : Self

Collected : 9/1/2026 4:19:00PM

A/c Status : P

Collected at : MR. RATAN LAL YADAV - FPSC MURLIPURA

E-129, BANK COLONY, MURLIPURA SCHEME

JAIPUR

Age : 32 Years

Gender : Female

Reported : 12/1/2026 10:10:55AM

Report Status : Final

Processed at : LPL-NATIONAL REFERENCE LAB

National Reference laboratory, Block E,

Sector 18, Rohini, New Delhi -110085



Test Report

Test Name	Results	Units	Bio. Ref. Interval
BETA 2 GLYCOPROTEIN 1, IgG SERUM (EIA)	1.93	SGU	<20.00

2006 International Consensus statement on Classification of definite APS

CLINICAL CRITERIA	LABORATORY CRITERIA
Arterial/Venous thrombosis	Cardiolipin antibodies (aCL)
Fetal Loss	Beta 2 Glycoprotein 1 antibodies
Premature birth	Lupus Anticoagulant (LA)

Note: APS is established if at least 1 Laboratory criteria and 1 Clinical criteria are met. The Laboratory criteria should be present on two or more occasions 12 weeks apart for diagnosing APS.

Comments

This test screens for the presence of IgG antibodies to Beta-2 glycoprotein which are associated with arterial and venous thrombosis and recurrent abortions.

The new International classification criteria for APS include Beta-2 glycoprotein for 2 key reasons namely:

- Beta-2 Glycoprotein antibodies maybe the only antibody present in 10% patients of Antiphospholipid syndrome (APS)
- Beta-2 Glycoprotein antibodies are highly specific for APS unlike Anticardiolipin antibodies which may be positive in certain infectious diseases

Indications

- Clinical history suggestive of Arterial or venous thrombosis
- History of Recurrent fetal loss /Intrauterine growth restriction/ Premature birth
- Thrombocytopenia

BETA 2 GLYCOPROTEIN 1, IgM (EIA)	5.77	SMU	<20.00
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2006 International Consensus statement on Classification of definite APS

CLINICAL CRITERIA	LABORATORY CRITERIA
Arterial/Venous thrombosis	Cardiolipin antibodies (aCL)
Fetal Loss	Beta 2 Glycoprotein 1 antibodies
Premature birth	Lupus Anticoagulant (LA)

Note



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Test Name	Results	Units	Bio. Ref. Interval
APS is established if at least 1 Laboratory criteria and 1 clinical criteria are met. The Laboratory criteria should be present on two or more occasions 12 weeks apart for diagnosing APS.			

Comments

This test screens for the presence of IgM antibodies to Beta-2 glycoprotein which are associated with arterial and venous thrombosis and recurrent abortions.

The new International classification criteria for APS include Beta-2 glycoprotein for 2 key reasons namely:

- Beta-2 Glycoprotein antibodies may be the only antibody present in 10% patients of Antiphospholipid syndrome (APS)
- Beta-2 Glycoprotein antibodies are highly specific for APS unlike Anticardiolipin antibodies which may be positive in certain infectious diseases

Indications

- Clinical history suggestive of Arterial or venous thrombosis
- History of Recurrent fetal loss/Intrauterine growth restriction/Premature birth
- Thrombocytopenia

CARDIOLIPIN ANTIBODY, IgG, SERUM (EIA)	8.08	GPL	<15.00
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Interpretation

RESULT IN GPL	REMARKS
<15	Negative
15-20	Equivocal
20-80	Low Positive
>80	High Positive

Comments

Anticardiolipin antibodies(ACA) belong to the group of Antiphospholipid antibodies which are positive in 30-40% cases of Systemic lupus erythematosus and also in patients with other Rheumatic diseases. Presence of cardiolipin antibodies is considered to be of significant diagnostic relevance in cases of Venous/Arterial thrombosis, Thrombocytopenia, Livedo reticularis, Habitual abortions and Neurological manifestations. Elevated ACA levels are also seen in patients with Cardiovascular insufficiency and



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Test Report

Test Name	Results	Units	Bio. Ref. Interval
Myocardial infarction. Results must be correlated with the history and clinical findings of the patient.			
CARDIOLIPIN ANTIBODY, IgM, SERUM (EIA)	9.94	MPL	<12.50

Interpretation

RESULT IN MPL	REMARKS
<12.50	Negative
12.50-20	Equivocal
20-80	Low Positive
>80	High Positive

Comments

Anticardiolipin antibodies (ACA) belong to the group of Antiphospholipid antibodies which are positive in 30-40% cases of Systemic lupus erythematosus and also in patients with other Rheumatic diseases. Presence of cardiolipin antibodies is considered to be of significant diagnostic relevance in cases of Venous/Arterial thrombosis, Thrombocytopenia, Livedo reticularis, Habitual abortions and Neurological manifestations. Elevated ACA levels are also seen in patients with Cardiovascular insufficiency and Myocardial infarction.



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Test Report

Test Name	Results	Units	Bio. Ref. Interval
PHOSPHOLIPID ANTIBODIES PANEL, IgG & IgM, SERUM (EIA)			
IgG	2.80	GPL U/mL	<12.00
IgM	1.89	MPL U/mL	<12.00

Interpretation

RESULT IN U/ml	REMARKS
< 12	Negative
12.00-18.00	Equivocal
>18.00	Positive

2006 International Consensus statement on Classification of definite APS

Clinical Criteria	Laboratory Criteria
Arterial/Venous thrombosis	Cardiolipin antibodies (aCL)
Fetal Loss	Beta 2 Glycoprotein 1 Antibodies
Premature birth	Lupus Anticoagulant (LA)

Note: APS is established if at least 1 Laboratory criteria and 1 Clinical criteria are met. The Laboratory criteria should be present on two or more occasions 12 weeks apart for diagnosing APS.

Comments

Phospholipid antibody is a quantitative assay to screen the presence of autoantibodies against cardiolipin, phosphatidyl serine, phosphatidyl inositol & phosphatidic acid in the diagnosis of an increased risk of thrombosis in patients with Systemic lupus erythematosus (20-35%) and other lupus like disorders.

This test is also used to diagnose Anti-phospholipid syndrome in patients with recent miscarriage (11-22% in all trimesters), Pulmonary hypertension, Non-vegetative endocarditis, Livedorecticularis, Stroke at young age and Deep vein thrombosis.

LUPUS ANTICOAGULANT BY DRVVT

(Nephelometric Clot Detection)

Patient Value	44.50	sec	30.02 - 42.60
Control Value	36.30	sec	
Screen Ratio	1.23		<1.20



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Test Report

Test Name	Results	Units	Bio. Ref. Interval
Patient Plasma : PNP (1:1) Mix Value	42.80	sec	30.02 - 42.60
Control Value	36.30	sec	
Mix Ratio	1.18	sec	<1.20
INTERPRETATION	No lupus like anticoagulant present		

Note

- As per ISTH guidelines Lupus Anticoagulant detection must be done by using at least two clot based assays employing separate clotting principles like PTT-LA & dRVVT.
- Results of this test should always be interpreted in conjunction with the patient's medical history, clinical presentation and other findings.
- A positive LA can be seen in otherwise normal individuals and in certain viral or other infections.
- Once a patient has been tested positive for LA, it is imperative that testing be repeated on a second occasion > 12 weeks after the initial testing.
- Anticoagulation therapy effects such as Warfarin (especially when the effect is supratherapeutic), excess Heparin, direct thrombin inhibitors (DTI) (eg, Dabigatran [Pradaxa]), Argatroban [Ancova], Bivalirudin [Angiomax]), direct factor Xa inhibitors (eg, Rivaroxaban [Xarelto], Apixaban [Eliquis], Edoxaban [Savaysa]) may result in a false-positive assay performance for LA. Clinical correlation and repeat testing after discontinuation (>1 week) of anticoagulation therapy is suggested.
- Although the dilute Russell viper venom time (DRVVT) reagents contain a heparin inhibitor (Polybrene) that is sufficient for neutralization of heparin (up to 1-2 U/mL), the results may not necessarily represent what would occur if no heparin were present in the specimen. Therefore, DRVVT results from heparinized plasma should be interpreted with caution.
- DRVVT assays, when performed in isolation, will not distinguish LA from heparin or inhibitors of factors V or VIII, which may cause false-positive results of LA testing.
- Test conducted on Citrated plasma.

Comments

Lupus Anticoagulants are heterogenous IgG or IgM autoantibodies which interfere with phospholipid dependent in vitro coagulation tests, particularly activated partial thromboplastin time (APTT). These antibodies are associated with thrombosis (arterial & venous), recurrent abortions, neurological & neuropsychiatric disorders.



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Various methods for testing Lupus Anticoagulants include PTT-LA, activated kaolin clotting time and dilute Russels Viper Venom time. Out of these the DrVVT assay is the most robust & specific because DrVVT is not influenced by deficiencies of intrinsic pathway or antibodies to factors VIII, IX or XI.



DMC - 115396

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Technical Director - Hematology &
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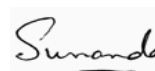
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-----End of report-----



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IMPORTANT INSTRUCTIONS

•Test results released pertain to the specimen submitted. •All test results are dependent on the quality of the sample received by the Laboratory. •Laboratory investigations are only a tool to facilitate in arriving at a diagnosis and should be clinically correlated by the Referring Physician. •Report delivery may be delayed due to unforeseen circumstances. Inconvenience is regretted. •Certain tests may require further testing at additional cost for derivation of exact value. Kindly submit request within 72 hours post reporting. •Test results may show interlaboratory variations. •The Courts/Forum at Delhi shall have exclusive jurisdiction in all disputes/claims concerning the test(s) & or results of test(s). •Test results are not valid for medico legal purposes. •This is computer generated medical diagnostic report that has been validated by Authorized Medical Practitioner/Doctor. •The report does not need physical signature.

(#) Sample drawn from outside source.

If Test results are alarming or unexpected, client is advised to contact the Customer Care immediately for possible remedial action.

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National Reference lab, Delhi, a CAP (7171001) Accredited, ISO 9001:2015 (FS60411) & ISO 27001:2013 (616691) Certified laboratory.

